Plan: PLAN-A | Drugs: Oxycodone | Version: 2026-01 | Effective: 2026-01-01

PLAN-A Pharmacy Policy: Short-Acting Opioid Analgesics

Overview

This policy explains how PLAN-A manages coverage of short-acting opioid pain medications. Its purpose is to support appropriate pain treatment for members while promoting safe prescribing, and to give prescribers and reviewers one consistent reference for the plan's expectations.

Oxycodone (immediate-release tablets) is covered with a quantity limit. Prior Authorization (PA) is required when a request exceeds the quantity limit, when the early-refill rule is not met, or when the member has an active prescription for another opioid.

Requests that fall within the quantity limit and meet every other rule in this policy are paid at the pharmacy without further review. When a request does need review, plan pharmacists and clinical reviewers apply the criteria below to the information submitted, and they may contact the prescriber for clarification.

Decisions are sent to the prescriber and the member in writing. A denial states the reason and the criteria applied, and it may be appealed through the plan's standard appeals process. Appeals are reviewed by a clinician who took no part in the first decision. The version of this policy in effect on the date a request is received is the one applied to that request.

Coverage Criteria

A request is considered for approval when the following conditions are met.

- The medication is for moderate to severe pain where non-opioid options are inadequate.

- The medication is not intended for long-term use without periodic review.

- Early-refill rule: a refill is not eligible until at least 75 percent of the previous days supply has elapsed. For a 30-day supply, this means no sooner than day 23.

- Overlapping opioid prescriptions are pended for prescriber review.

Reviewers also confirm that the member is actively enrolled on the date of the request, that the prescription is valid, and that the information supplied is consistent with the clinical notes. A request that conflicts with the documented treatment plan may be returned to the prescriber for correction before a decision is made.

A pended request stays open while the plan waits for the prescriber's response. The prescriber may reply by phone, fax, or the plan's electronic portal, and the review resumes once the reply is received.

Required Documentation

Each PA request must include the items below. A complete first submission is the fastest route to a decision, so prescriber offices should attach everything at the outset.

- Clinical notes with the pain diagnosis and a functional assessment.

- Prior non-opioid treatments tried and their outcomes.

- Confirmation the prescriber reviewed the state prescription monitoring program (PDMP) within the last 30 days.

- A treatment plan including expected duration.

- Prescriber name and NPI.

Incomplete requests are returned as "documentation incomplete". The return notice lists each missing item so the prescriber can supply it without resubmitting the whole request, and the review clock restarts when the missing items arrive.

Clinical notes should be recent enough to reflect the member's present condition and should be signed or otherwise authenticated by the treating clinician. Legible scanned or faxed copies are acceptable. The functional assessment should describe in plain terms what the member can and cannot do because of the pain, such as walking, sleeping, working, or caring for themselves, so that the reviewer can judge the need for treatment and later measure benefit.

Entries about prior treatments should name each therapy, give approximate start and stop dates, and describe the result, including any side effects or reasons the therapy was stopped. Where exact dates are unknown, the month and year are sufficient.

The PDMP confirmation may be a dated statement in the notes that the review took place and what it showed. Prescriber identification is used to confirm that the request comes from a licensed clinician who participates with the plan.

Step Therapy

No specific drug is required first. The member does not have to try and fail a named medication before oxycodone can be approved, and reviewers do not deny a request because a particular earlier drug was not used.

A documented trial of non-opioid pain management is required as part of the documentation above. Examples include acetaminophen or an NSAID, unless contraindicated. When a trial is not possible, the notes should state the contraindication or reason. The reviewer assesses whether the documented trial was reasonable for the member's situation.

Quantity Limits

The plan limits how much of the medication may be dispensed in a single fill period so that supplies match a standard course of treatment.

- Maximum 60 tablets per 30 days.

Higher requests need prior authorization with a documented clinical reason. The prescriber should explain in the clinical notes why the standard limit does not meet the member's needs and should state the exact quantity and the duration of the increase requested.

Reviewers evaluate each higher-quantity request individually. When the documentation supports it, an increase may be approved for a defined period and then return to the standard limit unless a new request is approved. Pharmacies that receive a quantity rejection should contact the prescriber, who can submit the request through the same channel used for any other PA.

Lost, stolen, or damaged supplies are handled through a separate review and are not governed by this limit. Members should speak with their pharmacy or the plan's member services line for help with those situations.

Renewal

Approvals last 6 months from the approval date. The approval notice states the exact end date, and prescribers are encouraged to start the renewal well before that date so that treatment is not interrupted.

Renewal needs updated clinical notes showing functional benefit, a current PDMP review, and an updated treatment plan. A renewal is reviewed against the criteria in effect on the date it is received, which may differ from those that applied to the original approval.

A renewal request that arrives after an approval has ended is treated as a new request, and the full documentation described earlier in this policy applies. A gap in coverage may occur while the new request is under review. Prescribers who are unsure whether a request counts as a renewal or a new request may ask the plan's pharmacy help desk before submitting.

This is a fictional policy created for software demonstration only.